

POWERED BY **ZAIZENS** · AN INTEGRATED HMS

PREMIUM · PROFESSIONAL EDITION

Inpatient Department (IPD)

Complete staff guide — admission, ward board, nursing care, medication, ward rounds, clinical discharge, and financial settlement.

PRODUCT

ZAIZENS Integrated HMS

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IPD User Manual

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Introduction

Welcome to the **ZAIZENS Integrated HMS IPD User Manual**. This document explains the full inpatient hospitalization lifecycle — from doctor admission order through bed assignment, daily care, and dual discharge (clinical + financial).

Key principle: A patient cannot physically leave until both **Clinical Discharge** (doctor) and **Financial Discharge** (cashier) are completed.

10

WORKFLOW STATIONS

3

DISCHARGE STAGES

8+

STAFF ROLES



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1. IPD Overview & Roles

The **Inpatient Department (IPD)** manages hospitalized patients from admission through discharge. All charges accumulate on a **running credit tab** after the admission deposit is collected.



Doctor

Admission decision, ward rounds, orders, clinical discharge, discharge summary



Cashier

Admission deposit, running bill settlement, financial discharge, IPD payment codes



Front Desk / ADT

Ward & bed assignment, transfers, confirm departure, bed released



Nurse / Nursing Aid

Receive patient, baseline vitals, drug chart administration, handover reports



Lab / Radiology

Process investigation orders; costs auto-added to running bill



Pharmacy

Ward dispensing, nursing supply requests, medication alerts

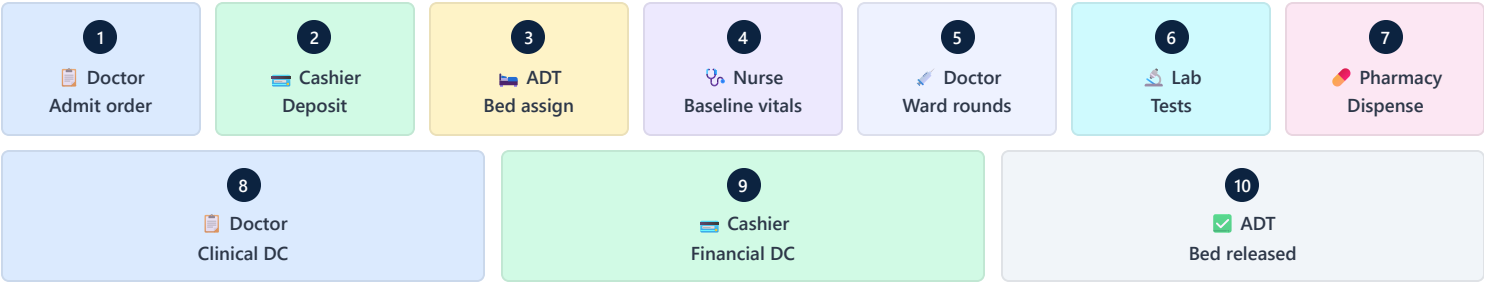
Key URLs

Screen	Path	Typical roles
IPD Hub	/ipd	All clinical staff
Ward board	/wards	ADT, Nurse, Doctor
Hospitalizations	/ipd/hospitalizations	Doctor, Admin
IPD medication	/ipd/medication	Doctor, Nurse
Drug chart	/ipd/chart/:admission_id	Nurse
Running bill	/ipd/running-bill/:id	Cashier, Doctor
IPD census	/ipd/census	Nursing, Admin
IPD settlement	/ipd/discharge	Cashier
Nurse handover	/ipd/handover	Nurse
Death registry	/ipd/death-registry	Doctor, Admin

Permissions: IPD modules use `adt.read` `adt.write` `nursing.write` `clinical.write` `ipd_medication.read` `cashier.write` .

2. Ten-Station IPD Workflow

The IPD workflow banner shows the inpatient journey through **10 stations** from admission order to bed release.



Doctor Admit Order → Cashier Deposit → ADT Bed Assignment → Nurse Baseline Vitals
→ Doctor Ward Rounds → [Lab / Radiology / Pharmacy orders on credit tab]
→ Doctor Clinical Discharge → Cashier Financial Discharge → ADT Bed Released

Step-by-step (all roles)

- 1 Doctor** — Decide hospitalization needed. Issue admission request with admitting diagnosis. `consult.write`
- 2 Cashier** — Collect admission deposit. Running credit tab opens for future charges. `cashier.write`
- 3 Front Desk / ADT** — Assign ward and bed. Bed status → Occupied. `adt.write`
- 4 Nurse** — Receive patient. Record baseline vitals. Execute care plan. `nursing.write`
- 5 Doctor** — Daily ward rounds. Update notes. Place new orders as needed.
- 6 Lab / Radiology** — Process orders. Upload results. Charges added to bill.
- 7 Pharmacist** — Dispense medications to ward. Costs on running bill.
- 8 Doctor** — Clinical Discharge when medically fit. Write discharge summary.
- 9 Cashier** — Present final bill. Settle all charges. Financial Discharge. Bed freed.
- 10 Front Desk / ADT** — Confirm patient departed. Bed → Available or Housekeeping.

Footnote: Deposit (Station 2) is required before bed assignment. Stations 8–10 must complete in order — clinical discharge before financial settlement.

3. Admission Statuses

Each hospitalization progresses through IPD status values visible on the ward board and census.

Status	Label	Meaning
ADMITTED	Active stay	Patient on ward; care and orders ongoing
CLINICAL DC	Awaiting financial DC	Doctor signed clinical discharge; cashier must settle
DISCHARGED	Discharged	Financial discharge complete; bed can be released

Bed statuses

Bed status	Display	Action
Available	Green — AVAILABLE	Admit new patient
Occupied	Purple border — patient name	Open chart, transfer, discharge flow
Housekeeping	Amber dashed — cleaning required	Mark ready when clean

4. IPD Hub & Navigation

The **Hospitalization (IPD) hub** at `/ipd` is the central dashboard for inpatient operations.

Hub KPIs

- **Active stays** — currently admitted patients
- **Beds available** — open beds across wards
- **Awaiting settlement** — clinical discharge done, financial pending
- **Completed** — all-time discharged count

Main menus (from hub)

Menu	Description
Hospitalizations	Active & completed inpatient stays list
Wards & beds	Visual bed board — admit, transfer, discharge
IPD medication	Treatments, drug chart, shift reports
Nurse handover	I-SBARR shift reports, doctor submission
Pharmacy requests	Nursing station supply requests
Ward rounds	Physician & nursing round notes
IPD census	Table of all current inpatients
Death registry	In-hospital mortality records
IPD settlement	Financial discharge & payment codes
Configuration	Surgery templates, checklists, buildings, OT

5. Admission Request

From consultation or chart

- 1 Doctor assesses patient needs hospitalization.
- 2 Open **Admit Patient** from consultation or patient chart.
- 3 Enter admitting diagnosis and reason for hospitalization.
- 4 Submit admission request — patient appears on **Awaiting bed assignment** list.

From Emergency / OPD

Stabilized emergency patients may transition to IPD: doctor issues admission request; ER charges may transfer to the IPD credit tab when configured.

Maternity

Maternity admissions link mother/neonatal charts. Labor patients may use pay-later deposit rules when enabled for emergency labor.

6. Deposit & Running Bill

Admission deposit

- 1 Cashier opens patient record or IPD settlement workspace.
- 2 Collect **admission deposit** as configured by hospital policy.
- 3 System opens **running credit tab** — all future IPD charges accumulate here.
- 4 Deposit amount shows on running bill and settlement screens.

Running bill

View at `/ipd/running-bill/:admission_id` :

- Itemized charge breakdown (consultation, ward, lab, radiology, pharmacy, misc)
- Deposit applied
- Balance due (payable before final invoice)
- Invoice forecast card for estimated totals

Adding charges

Doctors and authorized staff use **Add charge** modal: pick Service Catalog section, pharmacy inventory search, or miscellaneous line. Pharmacy charges may include clinical/dispensing instructions matching IPD prescription fields.

7. Ward Board & Bed Assignment

The **IPD Ward Board** at `/wards` provides a visual map of wards, beds, and active admissions.

Assign bed

- 1 Open ward board from IPD hub or `/wards`.
- 2 Review **Awaiting bed assignment** panel for pending admissions.
- 3 Click an **Available** bed or use + **Admit patient**.
- 4 Select patient and confirm — bed status → **Occupied**.

Transfer

Drag occupied bed to another bed/ward to transfer patient (when permissions allow). Transfer updates ward name and bed label on all IPD screens.

Manage beds (Admin)

Use **Manage beds** to add/remove wards and beds, set capacity, and mark beds out of service.

8. Nursing & Baseline Vitals

Receive patient on ward

- 1 Nurse opens occupied bed on ward board or IPD medication hub.
- 2 Record **baseline vitals**: BP, temperature, weight, SpO₂, pulse.
- 3 Review admitting diagnosis and doctor orders.
- 4 Begin care plan and medication schedule from drug chart.

IPD inbox

Nurses respond to clinical department alerts and IPD nurse message inbox at `/ipd/inbox` for pending orders and medication notifications.

Supply requests

Request drugs and ward materials via **Pharmacy requests** from the IPD hub.

9. Ward Rounds & Orders

Daily ward rounds

- 1 Doctor opens **Ward rounds** (`/ipd/ward-rounds`) or patient chart from bed board.
- 2 Review each inpatient: vitals, progress, active treatments.
- 3 Enter round notes and update clinical documentation.
- 4 Place new lab, radiology, pharmacy, or procedure orders — auto-added to running bill.

IPD treatment manager

At `/ipd/treatment/:admission_id` doctors start treatments, add prescriptions, and set dose schedules. Nurses administer via drug chart.

10. IPD Medication & Drug Chart

Full inpatient medication workflow at `/ipd/medication` and per-admission treatment pages.

Doctor — prescribe

- Start treatment with diagnosis and estimated duration
- Add prescriptions from pharmacy catalog or custom drug
- Dose slots generated automatically for nursing administration
- Extend, shorten, replace, or terminate treatment plans
- Administration protocol — review scheduled doses, add doctor comments

Nurse — drug chart

- Open **Bedhead drug chart** at `/ipd/chart/:admission_id`
- Confirm administration — turns green (locked)
- Mark missed/refused with reason (patient refused, vomited, unavailable)
- **Correct entry** for documentation errors (audited)

Shift reports

Nurse shift reports document treatments given during the shift. Handover board links to I-SBARR structured reports.

Audit trail

All dose changes, corrections, and administrations logged in treatment audit trail for clinical governance.

11. Nurse Handover

The **Nurse handover** module at `/ipd/handover` supports structured shift transition:

- I-SBARR format shift reports
- Treatments administered during shift
- Doctor submission of handover notes
- Outstanding tasks for incoming nursing team

Use at every nursing shift change on busy wards to maintain continuity of care.

12. Lab, Radiology & Pharmacy

During stay

- 1 Doctor places investigation orders from ward rounds or patient chart.
- 2 Lab/Radiology process orders — no separate prepayment; charges on credit tab.
- 3 Results upload to patient chart; clinicians notified via alerts.
- 4 Pharmacy dispenses to ward; nursing administers per drug chart.

IPD and Emergency orders generate clinical department alerts accessible from Lab/Radiology/Pharmacy sidebars when write permission is granted.

13. Clinical Discharge

Doctor signs clinical discharge

- 1 Confirm patient medically fit for discharge.
- 2 Open admission from ward board or hospitalizations list.
- 3 Write **discharge summary** with final diagnosis and instructions.
- 4 Sign **Clinical Discharge** — status → Awaiting financial DC.

Doctor-only: Only the admitting doctor (or authorized clinician) may sign clinical discharge unless bypass rules apply.

Bed shows **Tap to complete discharge** for cashier/ADT follow-up. Patient remains on ward until financial discharge.

14. Financial Discharge & Settlement

Cashier settlement

- 1 Open **IPD settlement** (/ipd/discharge) or Cashier IPD tab.
- 2 Locate patient — filter by name, ward, awaiting financial DC.
- 3 Review total charges, deposit, balance due.
- 4 Enter or validate **IPD payment code** from settlement workflow.
- 5 **Settle Bill** — issue receipt, mark Financial Discharge.
- 6 Bed status freed for housekeeping or next admission.

Zero balance

When deposit covers all charges, use **Zero • Confirm** to complete financial discharge without additional payment.

Refunds

Deposit exceeding final bill may trigger refund workflow per hospital policy.

15. IPD Census

IPD Census at `/ipd/census` lists all currently admitted patients with KPIs:

- Total inpatients / active / awaiting financial DC
- Average length of stay (LOS)
- Sortable table: patient, ward/bed, department, admitted date, LOS, diagnosis, doctor, bill, status

Use for nursing handoff, bed management meetings, and administration reporting.

16. Death Registry

Record in-hospital deaths via **Death registry** (</ipd/death-registry>) linked from IPD hub or patient chart when mortality occurs during admission.

- Document date, cause, and clinical context
- Coordinate with clinical and financial discharge workflows
- Supports statutory reporting where configured

17. Screen Reference (UI)

Illustrative panels matching live IPD React screens. Open `/ipd` and `/wards` while signed in for your deployment.

IPD Hub — /ipd

Hospitalization (IPD)

24
ACTIVE STAYS

18
BEDS AVAIL

3
AWAITING DC

Hospitalizations

Wards & beds

IPD medication

IPD census

Ward Board — /wards

General Ward · 12 beds

Bed A1
AVAILABLE

Bed A2
Jane Doe
Admitted 3d · Dr. Smith

Bed A3
Paul K.
AWAITING FIN. DC

Bed A4
Housekeeping

IPD Workflow Banner

1 Admit

2 Deposit

3 Bed

4 Vitals

5 Rounds

6 Lab

7 Rx

8 Clin DC

9 Fin DC

10 Released

Drug Chart — Nurse

Bedhead drug chart · AM doses

08:00	Paracetamol 500mg PO	✓ Given
12:00	Amoxicillin 500mg PO	Pending
18:00	Metoclopramide 10mg IV	Missed

Live screenshots: Sign in and open `/ipd` , `/wards` , or `/ipd/census` , then use Print → Save as PDF for site-specific training materials.

18. Troubleshooting

Issue	Solution
Cannot assign bed	Confirm cashier collected admission deposit; check awaiting assignment list
Bed still occupied after discharge	Complete financial discharge; mark housekeeping ready
Clinical discharge blocked	Only admitting doctor may sign; check outstanding orders
Running bill unexpected charges	Review charge breakdown; verify lab/rad/pharmacy orders
Nurse cannot administer dose	Treatment may be terminated; dose already given (locked)
Transfer failed	Target bed must be available; check permissions
IPD settlement not listed	Patient needs clinical discharge first
ER charges not on IPD bill	Verify ER-to-IPD charge transfer rules and admission link

19. Glossary

Term	Definition
IPD	Inpatient Department — hospitalized patients
ADT	Admission, Discharge, Transfer — bed and ward management
Running bill / credit tab	Cumulative charges during admission
Deposit	Up-front payment opening the credit tab
Clinical discharge	Doctor confirms patient medically fit to leave
Financial discharge	Cashier settles all charges; bed released
LOS	Length of stay — days since admission
Drug chart	Scheduled dose administration record for nurses
I-SBARR	Structured nursing handover format
Housekeeping	Bed cleaning state before next admission